

ASSAM MEDICAL COLLEGE AND HOSPITAL (AMCH)

Department of Neurology & Hydrocephalus Clinic

Barbari, Dibrugarh, Assam 786005

CONFIDENTIAL CLINICAL MEDICAL RECORD - NEUROLOGY CONSULTATION

Patient Name: Saikia, Debojit	MRN: AMCH-2026-7734	Sex: Male
DOB / Age: 19/09/1949 (Age: 76)	Date of Exam: 27/08/2026	Primary Caregiver: Rupiyoti Saikia (Son)
Attending Neurologist: Dr. Pranabesh Majumdar, MD, DM (Neurology)	Referring Physician: Dr. Nilotpal Das, MD (General Medicine)	Clinical Status: Outpatient Evaluation

REASON FOR EVALUATION

Comprehensive evaluation for classical triad of progressive magnetic gait difficulty, urinary urgency/incontinence, and subcortical psychomotor slowing over 10 months.

CLINICAL HISTORY (HISTORY OF PRESENT ILLNESS)

The patient is a 76-year-old Assamese male, retired railway engineer from Dibrugarh, presenting with a 10-month history of worsening gait instability, urinary urgency, and cognitive sluggishness. The family reports his feet appear 'stuck to the floor' when initiating walking. Over the last 5 months, he developed urinary urgency progressing to intermittent incontinence and subcortical mental slowing. Functional Assessment: Dependent for navigation and outdoor walking (requires two-person assistance or walker), dependent for medication administration and banking. Requires supervision for bathroom transfers and dressing due to gait instability. Behavioral Symptoms: Prominent apathy, psychomotor bradyphrenia, social withdrawal, but no nocturnal agitation or hallucinations.

PAST MEDICAL HISTORY	ACTIVE MEDICATIONS
- Hypertension for 15 years - Benign Prostatic Hyperplasia (on Tamsulosin 0.4 mg) - No prior meningitis, subarachnoid hemorrhage, or severe traumatic head injury	- Telmisartan 40 mg PO daily - Tamsulosin 0.4 mg PO bedtime - Aspirin 75 mg PO daily

PHYSICAL & NEUROLOGICAL EXAMINATION

Alert, oriented to self and place, notable psychomotor slowing. Cranial Nerves: Normal ocular motility, no nystagmus. Motor: Normal power 5/5 throughout; tone mildly paratonic in lower limbs, upper limbs normal. Reflexes: 2+ symmetric, plantars flexor. Gait: Classic 'magnetic' gait: wide base, short step height, reduced stride length, prominent hesitation on gait initiation, multi-step turn (5 steps for 180 degrees). Romberg positive.

STANDARDIZED COGNITIVE TESTING

Mini-Mental State Examination (MMSE): Score 21/30 | MoCA: Score 19/30

- Orientation: 8/10 (Disoriented to day of week, exact date)
- Registration: 3/3
- Attention & Calculation: 2/5 (Slow processing on Serial 7s)
- Recall: 2/3 (Recall readily elicited with semantic cues; retrieval deficit pattern)
- Language & Visuospatial: 6/9 (Significant psychomotor slowing; Clock Drawing mildly distorted; preserved basic naming)

DIAGNOSTIC WORKUP & NEUROIMAGING BIOMARKERS

- Laboratory: Normal metabolic, renal, liver, and thyroid panels. B12 normal (520 pg/mL).
- Brain MRI (Without Contrast): Marked symmetric ventricular enlargement out of proportion to sulcal widening (Evans Index = 0.36). Tightness of high convexity sulci with enlarged Sylvian fissures (Disproportionately Enlarged Subarachnoid Space Hydrocephalus - DESH sign). Periventricular halo of transependymal CSF flow. MTA Score: Grade 1. Fazekas Grade 1.
- High-Volume Lumbar Puncture (Tap Test): 40 mL CSF removed (Opening pressure normal: 140 mmH2O). Post-tap 10-meter walk test showed a 32% improvement in gait velocity and cadence.

DIAGNOSTIC IMPRESSION

- Idiopathic Normal Pressure Hydrocephalus (iNPH) with Secondary Neurocognitive Impairment (ICD-10: G91.2).
- Complete clinical triad (Hakim-Adams: gait apraxia, urinary incontinence, subcortical cognitive impairment) with positive imaging (DESH, Evans Index 0.36) and robust positive CSF tap test response.

TREATMENT PLAN & CLINICAL RECOMMENDATIONS

- Neurosurgical Consultation: Urgent referral to Neurosurgery team at AMCH for evaluation of Ventriculoperitoneal (VP) Shunt placement with programmable valve.
- Fall Prevention: Fall safety precautions, wheeled walker with supervision, removal of home loose rugs.
- Urological Management: Continue Tamsulosin; scheduled timed voiding protocol.
- Follow-Up: Post-operative neurology follow-up at 4 weeks post-VP shunt placement.

Electronically Verified & Signed by:

Dr. Pranabesh Majumdar, MD, DM (Neurology)
Professor & Unit Chief of Neurology | Reg No: AMC-17483
Assam Medical College & Hospital, Dibrugarh, Assam

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